

SESSION OBJECTIVES

At the conclusion of this session, the student should be able to:

1. Understand the common symptoms of each personality disorder and why they are assigned within their cluster
2. Develop an understanding of how personality disorders impact relationships
3. Apply this knowledge to clinical scenarios

Personality Disorders



-According to the DSM-5, a personality disorder is defined as a pattern of inner experience and behavior that deviates markedly from the expectations of the individual's culture.



-The pattern is enduring, inflexible, and pervasive across a broad range of personal and social situations.



-There are 10 identified personality disorders, which are categorized into 3 clusters, A, B, and C, based on descriptive similarities.



-Individuals may present with co-occurring personality disorders from different clusters.

Personality Disorder Markers

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-A personality disorder **must be present by early adulthood.**



-Most personality disorders **cannot be diagnosed in individuals under 18 years old**, unless the characteristics have been present for over a year.



-One exception is **antisocial personality disorder** which cannot be diagnosed until age 18, before 18 it is considered a conduct disorder.



-The diagnosis of a personality requires evaluation of one's long-term pattern of functioning over time and across different situations.



-The personality traits identified must be distinguished from personality traits which may emerge only during exceptionally stressful periods, exclusively during the onset of other mental health episodes, or co-occurring during an episode of substance abuse.

Personality Disorder Demographics

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-A clinician cannot accurately diagnose an individual with a personality disorder without **taking in account their ethnic, cultural and social background, as norms vary across cultures.** The clinician should utilize informants who are familiar with the individual's culture- family, friends.

-Some personality disorders have been **socially stereotyped by gender**, as an example, males are more frequently diagnosed with antisocial personality disorders and females with borderline, dependent and histrionic personality disorders. Clinicians must remain objective to avoid over generalizing based on this societal perception.

Cluster A Personality Disorders

HALLMARKS:

-Individuals appear “odd or eccentric”

-Related to the psychotic disorders- paranoid, schizoid and schizotypal

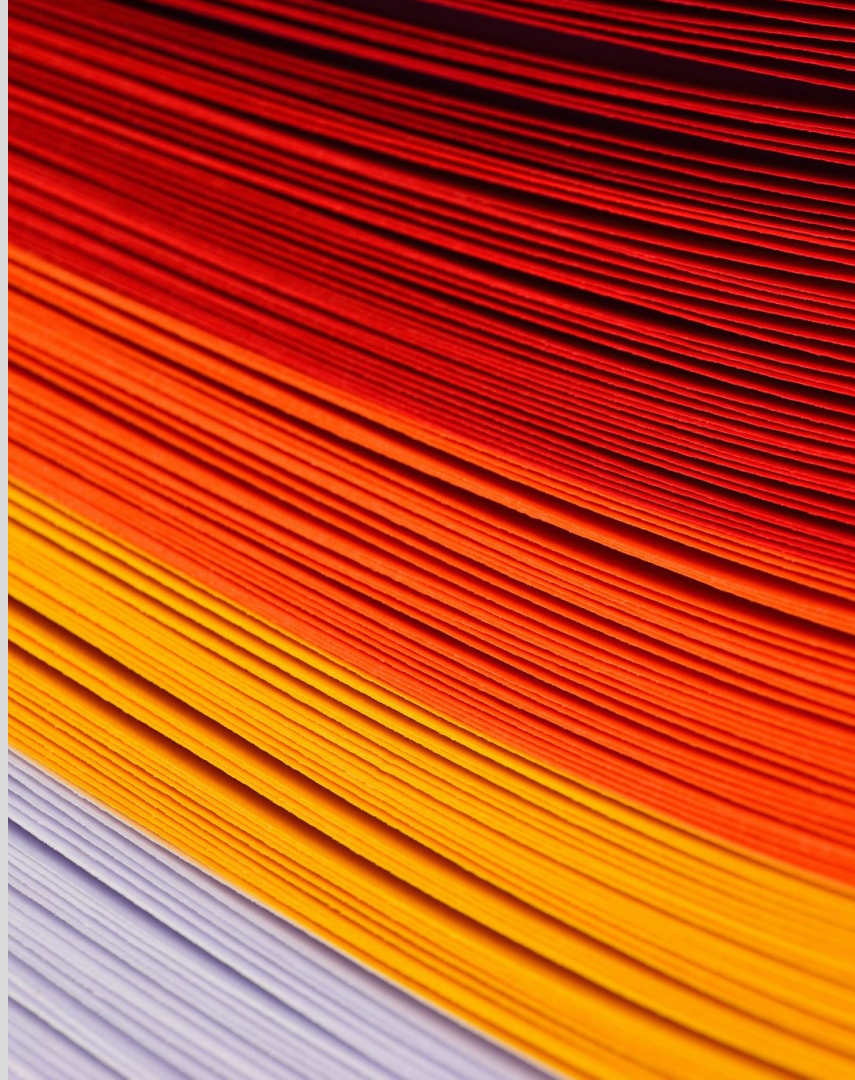
-Pattern of behavior does NOT occur exclusively during the course of a psychotic disorder(schizophrenia, bipolar disorder, or depression with psychotic features)

-If individual has a persistent mental disorder (i.e. Schizophrenia) and had a pre-existing personality disorder, it should also be documented as “premorbid”

Cluster B Personality Disorders

HALLMARKS:

- Dramatic
- Emotional
- Erratic
- Related to mood disorders, substance abuse and somatoform disorders
- Pattern of behavior does NOT occur exclusively during the course of a mood disorder, substance use disorder or somatoform disorder



Characteristics:

- Lack of conforming to social norms, ex. criminal behaviors
- Disregard for, violation of rights of others
- Dishonest
- Irresponsible
- Lack of remorse
- Diagnosis of conduct disorder onset before age 15

NOTE- colloquially referred to psychopaths or sociopaths

Antisocial Personality Disorder

Avoidant Personality Disorder

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Characteristics:

- Socially inhibited
- Feeling inadequate
- Overly sensitive to rejection or criticism
- Fear of being embarrassed or ridiculed
- Reluctant to take personal risks

NOTE-Individuals with avoidant personality disorder want to make friends but are hypersensitive to rejection, while individuals with schizoid personality disorder from cluster A, prefer to be alone and are indifferent to criticism or compliments.

Characteristics:

- Submissive and clingy behavior
- Fear of separation
- Difficulty making daily decisions
 - Seeks excessive reassurance
- Difficulty disagreeing for fear of loss of support
- Lack of confidence, so fears initiating ideas
- Fears being alone, due to unrealistic fear of being unable to care for themselves
- NOTE- Individuals in this category may end up in toxic relationships, manipulated, or taken advantage of.

Dependent Personality Disorder



Other Personality Disorders

Characteristics:

- Personality change due to the **direct effect** of a **medical condition**
- The personality observed is a change from the individual's previously typical personality pattern
- The change causes significant distress in functioning

Note – Certain Strep infections were associated with an increase in new OCD symptoms in some children. When treated with appropriate antibiotics, the OCD symptoms remitted. (**PANDAS**-**P**ediatric **A**utoimmune **N**europsychiatric **D**isorders **A**ssociated with **S**treptococcal infections.)

Summary

1- Each personality disorder has a set of common symptoms as well shared hallmark characteristics of their assigned cluster

2- Having a personality disorder can create significant strain on interpersonal relationships

Core References

1- Medical Knowledge and Its Application into Osteopathic Medical Practice

2- Osteopathic Patient Care

3- Interpersonal and Communication Skills in Osteopathic Medical Practice